This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Miconazole 2% cream for the treatment of Fungal Skin Infections, and Trimovate cream for inflamed intertrigo or infected eczema with a suspected secondary bacterial or candidal component

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Fungal Infections

Overview

- Fungal infections are usually caused by **dermatophytes**, **yeasts** (e.g. *Candida*), or **non-dermatophyte moulds**.
- Most are **superficial**, affecting **skin**, **nails**, or **mucosa**, and are generally **non-serious but can be persistent**.

**1. Tinea (Dermatophyte) Infections

Tinea Corporis (body), Cruris (groin), Pedis (foot), Manuum (hand):

- **First-line**: **Topical antifungals** (e.g. **clotrimazole**, **terbinafine**, **ketoconazole**), apply **once or twice daily for 2–4 weeks**.
- Use **oral antifungals** (e.g. **terbinafine**, **itraconazole**) if:
    - Extensive/severe infection
    - Failed topical therapy
    - Immunosuppression

Tinea Capitis (scalp):

- Requires **oral antifungal treatment**:
    - **Terbinafine** (first-line) or **griseofulvin** (children)
    - Adjunct **medicated antifungal shampoo** (e.g. ketoconazole) to reduce spread
- Consider **referral** for diagnosis confirmation and contact tracing in children (especially in schools)

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2. Candidiasis (Yeast Infections)

Oral Candidiasis:

- First-line: **Topical nystatin** or **miconazole gel**
- **Fluconazole** orally if:
    - Severe
    - Unresponsive to topical
    - Immunocompromised

Vaginal Candidiasis:

- **Topical imidazole** (e.g. clotrimazole) or **oral fluconazole 150 mg single dose**
- Recurrent: Consider **fluconazole 150 mg every 72 hours for 3 doses**, then weekly for 6 months

Intertrigo or Skin Fold Candidiasis:

- **Topical antifungal with or without mild corticosteroid** (e.g. miconazole/hydrocortisone)
- Address underlying moisture or friction

When to Refer

- Severe, widespread, or resistant infections
- Immunocompromised or diabetic patients
- Diagnostic uncertainty or suspected non-fungal skin condition

General Advice

- Continue treatment for **at least 1–2 weeks after resolution**.
- Keep skin clean and dry.
- Avoid sharing towels, socks, footwear.
- Consider screening or treating close contacts in tinea capitis or extensive infection.

Patient Group Direction

for the supply/administration of

Miconazole 2% cream

for the treatment of

Fungal Skin Infection

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Miconazole 2% cream is indicated for the treatment of superficial fungal skin infections such as athlete's foot, ringworm, and candidal intertrigo in individuals aged 16 years and over.
Inclusion criteria	- Individuals aged 16 years and over. - Diagnosis of superficial fungal skin infection (e.g. athlete's foot, ringworm). - Informed consent obtained. - No contraindications to miconazole.
Exclusion criteria	- Known hypersensitivity to miconazole or any of the excipients. - Infected, broken, or oozing skin where topical antifungal is inappropriate. - Nail or scalp infections (requires systemic or alternative treatment). - Pregnancy or breastfeeding. Refer to the GP.
Cautions	- Avoid contact with eyes and mucous membranes. - Continue treatment for at least one week after all signs and symptoms have resolved. - Wash hands before and after application. - Counsel on hygiene to prevent reinfection or spread. - Caution should be exercised and anticoagulant effect should be monitored in patients on warfarin or other vitamin K antagonists.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Miconazole 2% cream
Legal category	P (Pharmacy medicine). Where the P licence is narrower than this PGD, the P licence governs.
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Topical application to clean, dry affected area twice daily. Rub the cream into the skin with your finger until it has fully penetrated.
Dose and frequency	Apply thinly to affected area twice daily. The duration of therapy can vary depending on the localisation and the severity of the lesion, but treatment should be continued for at least one week after disappearance of all signs and symptoms.
Quantity to be administered and/or supplied	One 30 g tube per treatment course.
Maximum or minimum	Minimum 2 weeks; continue for at least one week after all signs and

treatment period	symptoms have resolved. Maximum 4 weeks.
Adverse effects	Burning, stinging, itching, redness. Rare: allergic reaction, contact dermatitis. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form, route and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Wear loose-fitting clothes made of cotton or material designed to keep moisture away from the skin. Maintain good hygiene and wash affected skin areas daily and dry thoroughly, especially in the skin folds. Avoid scratching affected skin, as this may spread the infection to other sites. Do not share towels, and wash towels, clothes and bed linen frequently to eradicate fungal spores and reduce the risk of transmission.

Key references

● NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017

https://www.nice.org.uk/guidance/mg2

- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Trimovate cream

for the treatment of

Inflammatory Fungal Skin Conditions

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Trimovate cream is indicated for short-term treatment of inflamed intertrigo, infected eczema or seborrhoeic dermatitis where a secondary bacterial or candidal component is suspected and combined topical therapy is appropriate. It is NOT a treatment for ringworm (tinea of any site), athlete's foot or any other primary fungal infection: those are treated with miconazole under the first arm of this document, or referred.
Inclusion criteria	- Adults aged 18 years and over. - Localised inflamed intertrigo, infected eczema or seborrhoeic dermatitis with a suspected secondary bacterial or candidal component (primary fungal infection excludes; see exclusion criteria). - No signs of systemic infection. - Informed consent obtained.
Exclusion criteria	- Known hypersensitivity to corticosteroids, nystatin, oxytetracycline, or any excipients. - Skin infections requiring systemic therapy. - Broken, ulcerated, or weeping skin. - Use on face, genitals, or long-term use without review. - Pregnancy or breastfeeding. Refer to the GP. - Primary fungal infection of the skin (tinea of any site, including ringworm and athlete's foot), primary bacterial skin infection (impetigo, cellulitis), or viral skin infection (herpes simplex, chickenpox, shingles, warts, molluscum). Trimovate is contraindicated in all of these (SmPC 4.3).
Cautions	- Do not use under occlusion. - Avoid prolonged use; assess if symptoms persist or worsen. - Avoid contact with eyes and mucous membranes. - If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Trimovate cream containing clobetasone 17-butyrate 0.05% w/w, oxytetracycline 3.0% w/w as calcium oxytetracycline and nystatin 100,000 units per gram.
Legal category	POM
Storage	Store below 25°C.
Route/method of	Topical application to affected area once or twice daily.

administration	Apply thinly and gently rub in using only enough to cover the entire affected area. Wash hands after applying unless it is the hands that are being treated. Allow adequate time for absorption after each application before applying an emollient if needed.
Dose and frequency	Apply thinly to affected area once or twice daily as directed.
Quantity to be administered and/or supplied	One 30 g tube per treatment course.
Maximum or minimum treatment period	Maximum 7–10 days continuous use without review. If the infection worsens, treatment and diagnosis should be re-evaluated as soon as possible. If the condition does not improve within seven days, treatment and diagnosis should be re-evaluated.
Adverse effects	Skin thinning, irritation, burning, itching. Rare: allergic reaction, superinfection. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form, route and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to	Supply the patient information leaflet (PIL) provided with the medication.

be given to patient or carer	
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen, do not improve within seven days, or they become systemically very unwell. Do not use for more than 7 to 10 days without review. Wash the affected skin daily and dry thoroughly, especially in skin folds. This helps to reduce the risk of reinfection. Wash clothes and bed linen frequently. Avoid sharing towels and wash them regularly. Wear loose-fitting clothes made of cotton or materials designed to keep moisture away from the skin. Avoid scratching the affected skin as this may spread the infection.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
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Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

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This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Miconazole exclusion criteria: pregnancy or breastfeeding is an exclusion with referral; the unless-approved-by-prescriber wording is removed because there is no prescriber in a PGD supply.

Miconazole cautions and treatment-period row: the post-resolution rule now reads at least one week after all signs and symptoms have resolved in every place it appears, matching the dose row.

Miconazole legal category: narration about why the P arm is included was removed; the P-licence-governs rule stays.

Trimovate inclusion criteria: the indication's own conditions are named (inflamed intertrigo, infected eczema or seborrhoeic dermatitis with a secondary component) and the clash with the primary fungal infection exclusion removed.

Trimovate exclusion criteria: pregnancy or breastfeeding is an exclusion with referral; the unless-deemed-appropriate-by-prescriber wording is removed.

Trimovate follow-up advice: the 3 to 4 week template line is replaced with the arm's own seven-day and 7 to 10 day rules.

Trimovate follow-up advice: the sentence about eradicating the fungus was removed because Trimovate does not treat primary fungal infection.

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Legal category corrected: miconazole 2% cream is a P medicine, not a POM; the arm is retained so the supply is assessed and recorded to the PGD standard. 'Unless advised by prescriber' removed from the Trimovate cautions: there is no prescriber in a PGD supply.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is

removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

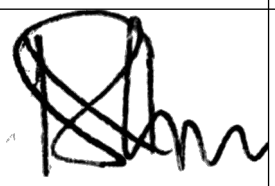
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Trimovate arm: the previous version framed Trimovate as a treatment for fungal skin infection and sat it under a guidance summary about tinea. The Trimovate SmPC contraindicates primary fungal, bacterial and viral skin infection. The indication now names inflamed intertrigo, infected eczema and seborrhoeic dermatitis with a suspected secondary component, states that Trimovate is not a treatment for ringworm or athlete's foot, and primary fungal, bacterial and viral infection are exclusions. The document title says which arm treats which.

Signed on behalf of Get Real Health

Version v006, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.